

C807

Healthcare Compliance

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	_____	

Items

1.

A 300-bed hospital that won a regional quality award last spring is building its coding compliance program. The HIM director has written a detailed coding policy manual and posted it on the intranet, which was redesigned this year. Nothing else has been set up yet. Based on OIG compliance program guidance, what is the most important gap?

- A. The manual should be printed in binders instead of posted on the intranet.
- B. The manual alone is not a program; it also needs an accountable compliance officer, training, a reporting line, auditing and monitoring, discipline, and corrective action.
- C. There is no gap; a written policy manual satisfies OIG guidance.
- D. The hospital needs a malpractice insurance review.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2.

A clinic billing manager who has worked there for eleven years notices that one physician's claims almost always use the highest-level office visit code. She decides not to look into it because the physician is the clinic's top earner. If the claims turn out to be false, which statement best describes the clinic's exposure under the False Claims Act?

- A. Exposure is lower because the physician brings in the most revenue.
- B. No liability, because the FCA requires proof of specific intent to defraud.
- C. Liability can attach, because deliberate ignorance or reckless disregard meets the "knowingly" standard even without intent to defraud.
- D. Only the physician can ever be liable; the clinic never is.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3.

A coder at a busy outpatient surgery center with a new EHR reviews a case where the surgeon performed a procedure described by one comprehensive CPT code. The charge sheet lists each component step of that procedure as a separate code. The center's payer mix is 60% Medicare. What compliance problem does the charge sheet show?

- A. Upcoding.
- B. No problem, because every step was actually performed.
- C. Downcoding.
- D. Unbundling.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4. [TRANSFER — reversed stem / different variable]

A coder assigns a level 5 emergency department visit code when the documentation supports only level 3. What is this practice called?

- A.** Unbundling.
- B.** Downcoding.
- C.** Modifier misuse.
- D.** Upcoding.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5.

A physician owns 20% of an imaging center across town that recently added an MRI unit. She refers her Medicare patients there for MRI scans, and the arrangement fits no exception. The referrals are medically necessary and the patients report high satisfaction. Which law is most directly implicated, even without proof of intent?

- A.** The HIPAA Privacy Rule.
- B.** The Anti-Kickback Statute, which applies only once intent is proven.
- C.** The Stark Law (physician self-referral).
- D.** No law, because the scans were medically necessary.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6. [TRANSFER — reversed stem / different variable]

A lab pays a nurse practitioner \$50 for each Medicare patient she sends there for testing, and investigators can show the payment was meant to induce referrals. Which law fits best?

- A.** The Anti-Kickback Statute.
- B.** Neither, because small per-patient payments are always allowed.
- C.** The Stark Law.
- D.** EMTALA.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7.

An inpatient coder at a teaching hospital known for its cardiology program sees lab results showing low potassium and orders for potassium replacement. The physician never documented hypokalemia or any related diagnosis. The case was discharged on a weekend. What should the coder do?

- A. Send a compliant, non-leading query asking the physician to clarify the clinical significance of the findings.
- B. Send a query saying "Please document hypokalemia so we can capture the higher payment."
- C. Never query; code only exactly what is written.
- D. Code hypokalemia from the lab values and treatment.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8.

A physician group with offices in three counties finishes a good-faith internal review and, on March 1, identifies that Medicare overpaid it \$18,400 over two years. The group's billing vendor is switching software next quarter. By what date must the overpayment be reported and returned?

- A. Only when Medicare sends a demand letter.
- B. With the next annual cost report.
- C. After the software switch next quarter.
- D. April 30.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9. [TRANSFER — reversed stem / different variable]

A provider returned an overpayment on June 14, exactly on its 60-day deadline. On what date did it identify the overpayment?

- A. June 14.
- B. April 15.
- C. May 15.
- D. August 13.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10.

The HIM director at a critical access hospital, who is also finishing a master's degree, wants to find coding errors before claims leave the building so that incorrect claims are never paid. The hospital's last external audit was two years ago. Which audit type fits this goal?

- A.** A Recovery Audit Contractor (RAC) review.
- B.** A repeat of the external audit from two years ago.
- C.** A prospective (pre-bill) audit.
- D.** A retrospective audit.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11.

An internal coding audit at a regional health system that recently merged with a rural clinic reviewed 80 inpatient records and found 6 with coding errors. The auditor has 15 years of experience. The system's benchmark is 95% accuracy. What is the coding accuracy rate, and does it meet the benchmark?

- A.** 92.5%; it falls short of the benchmark.
- B.** 94%; it falls short of the benchmark.
- C.** 81.25%; it falls short of the benchmark.
- D.** 7.5%; it meets the benchmark.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

12. [TRANSFER — reversed stem / different variable]

An audit of 120 claims must show at least 95% accuracy. What is the maximum number of claims that can contain errors?

- A.** 0, because any error fails a compliance audit.
- B.** 6
- C.** 5
- D.** 114

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer B Concept 1

A 300-bed hospital ~~that won a regional quality award last spring~~ is building its coding compliance program. The HIM director has written a detailed coding policy manual and posted it on the intranet, ~~which was redesigned this year~~. Nothing else has been set up yet. Based on OIG compliance program guidance, what is the most important gap?

Target word: what a compliance program needs beyond policies **Decides:** Only one of the seven elements (written policies) exists.

A	Camouflage	Reacts to the intranet detail; the format of the manual does not decide the gap.
B	Correct	OIG guidance describes seven elements; policies are only the first.
C	Absolute language	A policy nobody trains on, monitors, or enforces is not an effective program.
D	Adjacent category	Malpractice coverage is risk management, not a compliance program element.

2. Answer C Concept 2

A clinic billing manager ~~who has worked there for eleven years~~ notices that one physician's claims almost always use the highest-level office visit code. She decides not to look into it because ~~the physician is the clinic's top earner~~. If the claims turn out to be false, which statement best describes the clinic's exposure under the False Claims Act?

Target word: FCA knowledge standard **Decides:** Choosing not to look at a red flag is deliberate ignorance, which meets "knowingly."

A	Camouflage	Revenue contribution has no bearing on liability.
B	Confusion set	Confuses the civil FCA standard with criminal fraud intent; the FCA says no specific intent is required.
C	Correct	The FCA defines knowingly to include deliberate ignorance and reckless disregard.
D	Absolute language	The entity that submits or causes false claims can be liable.

3. Answer D Concept 3

A coder at a busy outpatient surgery center ~~with a new EHR~~ reviews a case where the surgeon performed a procedure described by one comprehensive CPT code. The charge sheet lists each component step of that procedure as a separate code. ~~The center's payer mix is 60% Medicare~~. What compliance problem does the charge sheet show?

Target word: name the billing error **Decides:** Components billed separately when one comprehensive code covers them.

A	Confusion set	Upcoding reports a higher level of service than documented; the issue here is splitting, not level.
B	Dropped element	Ignores the rule that a comprehensive code already includes its components.
C	Inverted operation	Downcoding under-reports; separate component billing usually over-reports.
D	Correct	Reporting components separately instead of the comprehensive code is unbundling.

4. Answer D Concept 3 • transfer

A coder assigns a level 5 emergency department visit code when the documentation supports only level 3. What is this practice called?

Target word: name the error from the reverse direction **Decides:** Higher level reported than documented.

A	Confusion set	Unbundling splits a comprehensive service into parts; this is one code at the wrong level.
B	Inverted operation	The reported level is higher, not lower, than documented.
C	Adjacent category	No modifier is involved; the visit level itself is wrong.
D	Correct	Reporting a higher level than the record supports is upcoding.

5. Answer C Concept 4

A physician owns 20% of an imaging center across town ~~that recently added an MRI unit~~. She refers her Medicare patients there for MRI scans, and the arrangement fits no exception. ~~The referrals are medically necessary and the patients report high satisfaction.~~ Which law is most directly implicated, even without proof of intent?

Target word: which fraud-and-abuse law **Decides:** Physician financial interest + designated health service + no exception + no intent needed.

A	Adjacent category	HIPAA governs health information, not referral relationships.
B	Confusion set	AKS requires intent; the stem asks for the law that applies without proof of intent.
C	Correct	Stark is strict liability for physician referrals of designated health services to an entity with which the physician has a financial relationship.
D	Camouflage	Medical necessity does not cure a Stark violation.

6. Answer A Concept 4 • transfer

A lab pays a nurse practitioner \$50 for each Medicare patient she sends there for testing, and investigators can show the payment was meant to induce referrals. Which law fits best?

Target word: which law — reversed facts **Decides:** Payment intended to induce federal program referrals from a non-physician.

A	Correct	AKS prohibits knowingly paying anyone to induce referrals of federal program business.
B	Absolute language	Any remuneration meant to induce referrals can violate AKS regardless of size.
C	Confusion set	Stark covers physician (and immediate family) financial relationships, not payments to a nurse practitioner.
D	Adjacent category	EMTALA governs emergency screening and stabilization, not referral payments.

7. Answer A Concept 5

An inpatient coder at a teaching hospital ~~known for its cardiology program~~ sees lab results showing low potassium and orders for potassium replacement. The physician never documented hypokalemia or any related diagnosis. ~~The case was discharged on a weekend.~~ What should the coder do?

Target word: next step when indicators lack a diagnosis **Decides:** Clinical indicators exist but no provider documentation.

A	Correct	Coders code from provider documentation and clarify gaps with non-leading queries.
B	Confusion set	That is a leading query tied to reimbursement, which compliant query practice forbids.
C	Absolute language	Queries are the accepted way to resolve conflicting or incomplete documentation.
D	Dropped element	Skips the provider documentation requirement.

8. Answer D Concept 6

A physician group ~~with offices in three counties~~ finishes a good-faith internal review and, on March 1, identifies that Medicare overpaid it \$18,400 over two years. ~~The group's billing vendor is switching software next quarter.~~ By what date must the overpayment be reported and returned?

Target word: deadline for returning an identified overpayment **Decides:** 60 days from the March 1 identification date.

A	Inverted operation	The provider must act on its own identification; it does not wait for the payer.
B	Adjacent category	The cost-report timing applies only when an overpayment is tied to a cost report; this is a claims overpayment.
C	Camouflage	Vendor changes do not extend the 60-day deadline.
D	Correct	March 1 + 30 days = March 31; + 30 more = April 30 (60 days).

9. Answer B Concept 6 • transfer

A provider returned an overpayment on June 14, exactly on its 60-day deadline. On what date did it identify the overpayment?

Target word: solve for identification date **Decides:** Count back 60 days from June 14.

A	Wrong stem number	That is the return date given in the stem.
B	Correct	April 15 + 15 = April 30; + 31 = May 31; + 14 = June 14 (60 days).
C	Confusion set	Uses a 30-day window, not the 60-day rule.
D	Inverted operation	Adds 60 days instead of counting back.

10. Answer C Concept 7

The HIM director at a critical access hospital, ~~who is also finishing a master's degree,~~ wants to find coding errors before claims leave the building so that incorrect claims are never paid. ~~The hospital's last external audit was two years ago.~~ Which audit type fits this goal?

Target word: audit timing **Decides:** "Before claims leave the building."

A	Adjacent category	RAC reviews are external and post-payment.
B	Camouflage	Reacts to the old-audit detail; it is still a post-payment look back.
C	Correct	Pre-bill review corrects errors before the claim is submitted and paid.
D	Inverted operation	Retrospective review happens after payment, so errors have already been paid.

11. Answer A Concept 8

An internal coding audit at a regional health system that recently merged with a rural clinic reviewed 80 inpatient records and found 6 with coding errors. The auditor has 15 years of experience. The system's benchmark is 95% accuracy. What is the coding accuracy rate, and does it meet the benchmark?

Target word: accuracy rate vs. benchmark **Decides:** 74 correct of 80 reviewed.

A	Correct	$(80 - 6) / 80 = 74 / 80 = 92.5\%$, below 95%.
B	Dropped element	Subtracts 6 from 100 without dividing by the 80 records.
C	Camouflage	Uses the auditor's 15 years: $(80 - 15) / 80$.
D	Inverted operation	7.5% is the error rate, not the accuracy rate.

12. Answer B Concept 8 • transfer

An audit of 120 claims must show at least 95% accuracy. What is the maximum number of claims that can contain errors?

Target word: solve for allowable errors **Decides:** 5% of 120.

A	Absolute language	The benchmark allows up to 5% errors.
B	Correct	$120 \times 0.05 = 6$ errors allowed (114 correct = 95%).
C	Wrong stem number	Uses the percentage as the count.
D	Inverted operation	That is the minimum number of correct claims, not errors.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	3	
Wrong stem number	A figure copied from the prompt that is not the answer.	2	
Adjacent category	The right idea in the wrong bucket.	6	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	7	
Absolute language	Always, never, guarantee — where judgment applies.	5	
Confusion set	Every option is real; the test is exact role-matching.	7	
Camouflage	Answers a stem fact that was never the question.	6	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Elements of an effective compliance program	OIG guidance calls for seven working elements: written policies, a compliance officer and committee, training, open lines of reporting, auditing and monitoring, consistent discipline, and prompt corrective action.	Believing a written policy manual is the compliance program.	An organization has one element in place and asks what is missing.
2	False Claims Act knowledge standard	“Knowingly” includes actual knowledge, deliberate ignorance, and reckless disregard; specific intent to defraud is not required.	Thinking liability needs proof that someone meant to cheat.	Someone notices a billing red flag and chooses not to look.
3	Upcoding vs. unbundling	Upcoding reports a higher level than documented; unbundling reports parts separately when one comprehensive code covers them.	Using the two terms interchangeably.	“One comprehensive code” vs. “higher level than documented.”
4	Anti-Kickback Statute vs. Stark Law	AKS is an intent-based criminal law against paying anyone to induce federal program referrals; Stark is a strict-liability civil law against physician self-referral for designated health services to entities they have a financial relationship with.	Assuming both laws require intent, or that Stark covers any referrer.	A physician’s ownership stake (Stark) vs. a payment meant to induce referrals (AKS).
5	Code only from provider documentation; query properly	Codes come from provider documentation; when clinical indicators suggest an undocumented condition, send a compliant, non-leading query.	Coding a diagnosis from lab values alone, or wording a query that leads to a paying answer.	Clinical evidence in the chart with no matching provider diagnosis.
6	60-day overpayment rule	An identified Medicare or Medicaid overpayment must be reported and returned within 60 days of identification; keeping it creates False Claims Act exposure.	Waiting for the payer to ask for the money back.	A date the overpayment was identified and a question about the deadline.
7	Prospective vs. retrospective audits	Prospective (pre-bill) audits catch errors before claims go out; retrospective audits review claims after payment, as external RAC audits do.	Thinking a post-payment review prevents incorrect payments.	“Before the claim is sent” vs. “after payment.”
8	Coding accuracy rate	Accuracy rate = records coded correctly / records reviewed; compare it to the benchmark (commonly 95%).	Reporting the error rate as the accuracy rate, or subtracting the error count from 100.	A number of records reviewed and a number with errors.

Course scope (public description): Healthcare Compliance examines the role of the coding professional within healthcare information management. The course covers compliance plans, issues that arise with noncompliance, and management of internal and external audits.